Royal Devon University Healthcare NHS Foundation Trust

Clinical Guideline for:
Care of Women Who Decline Recommended Maternity Care

Eastern and Northern Maternity Services

Summary

This guideline aims:

- To provide health professionals with the appropriate information to ensure there is a consistent approach regarding provision of information, care and documentation for women who decline care according to local Trust guidance.
- To provide health professionals with appropriate pathways via which to seek support in situations which may be challenging.
- To provide specific guidance for midwives to follow when routine care is declined.

Key Points

- The role of the midwife is to provide safe, evidence based care to women and their babies. However, this role also includes being an advocate for women and even when women make choices that are not in line with Trust or national guidance, these must be respected.
- **Women should be given the risks, benefits and recommendations for any plan of care or intervention in a non-threatening way, in order for them to make an informed choice.**
- **All staff** have a responsibility to ensure that all women are provided with consistent evidence based advice in which to make an informed choice
- **Midwifery Managers /Matrons** who are made aware of women declining recommended care should ensure that there is a clear plan of support put in place prior to the birth, a copy of which should be sent to the woman.
- **Midwives:** If a woman declines your advice regarding her care, you should seek further guidance from your line manager to ensure that all possibilities have been explored and that the outcome is appropriately documented. The woman should be offered the opportunity to read what has been documented about the advice she had been given. She may sign this if she wishes.

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1. INTRODUCTION

1.1 Midwives and obstetricians must provide clear, evidence based information to women during pregnancy, birth and the postnatal period in order to for women to be able to make informed choices regarding their care.

1.2 These health professionals work with a range of evidence based guidelines to assist them with providing this information and planning care with women. The local guidelines for maternity services are accessible to midwives and other professionals via the HUB. There is also a range of Patient Information Leaflets to assist with the provision of information to women and their families which are available in print and via the HUB also.

1.3 There will be times when women choose not to accept care according to these guidelines which may be challenging for the health professionals involved.

2. PURPOSE

This guideline aims to:

2.1 Provide health professionals with the appropriate information to ensure there is a consistent approach regarding provision of information, care and documentation for all women, especially those who choose not to receive care in accordance with Trust guidelines.

2.2 Provide health professionals with appropriate pathways via which to seek support in situations which may be challenging.

2.3 Provide specific guidance for midwives to follow when routine care is declined.

3. DEFINITIONS

3.1 There are a number of circumstances where a woman may decline routine care. This may occur antenatally, during labour or in the postnatal period. The response from health professionals will depend on the specific situation and the woman's individual circumstances and preferences.

3.2 If a woman has capacity, she is entitled to make choices regarding her care during pregnancy and labour, even if health professionals have concerns that those choices may cause harm to her unborn baby. Once the baby is born, health professionals have a duty of care to the neonate that may override parental choice. (See Appendix 1).

4. DUTIES AND RESPONSIBILITIES OF STAFF

4.1 All staff have a responsibility to ensure that all women are provided with consistent evidence based advice in order to make informed choices. Where women decline routine care, it is important to ascertain that they have fully understood the choices they are making. Women may need time to make their final choices about some issues and written information should be provided where available. It is important that health professionals appreciate that everyone considers risk, benefits and consquences in different ways due to their background, experience and beliefs and that these differences must be respected.

4.2 Obstetricians, Midwifery Managers or Midwifery Matrons who are made aware of women declining recommended normal care should ensure that, when possible, there is a plan of support put in place prior to the birth, a copy of which should be sent to the woman.

5. SPECIFIC RESPONSIBILITIES OF HEALTH PROFESSIONALS AND ACTION REQUIRED WHEN PROVIDING CARE:

5.1 Pregnancy

5.1.1 Midwife providing care

The midwife seeing the woman in pregnancy (ideally her named midwife) should discuss the recommended plan of care with her. If the woman chooses to decline the recommended care, the midwife should discuss this with her in a non-judgemental manner, explaining any potential risks and exploring alternative options for care.

Factual information and, where possible, simple statistics should be provided to help the woman understand the level of risk, for example a 1 in 200 chance of uterine rupture when planning a VBAC (RCOG, 2015). Absolute numbers should be used rather than relative risk. Both positive and negative framing should be used to help women to understand the risks or benefits of actions, for example, a 3 in 100 chance of a treatment being unsuccessful means a 97 in 100 chance of it being successful.

Where possible, written information should be given, or women should be signposted to reliable sources of information, for example Trust Patient Information Leaflets.

All discussions should be clearly documented within the Electronic Patient Record (EPR) and the women will have the opportunity to read what has been recorded.

Midwives should keep in mind that the Code (NMC) states that midwives must 'respect, support and document a person's right to refuse care and treatment'.

If the midwife feels that there is significant risk to the woman or her baby in the light of the choices she has made, these should be discussed with her clinical midwifery manager. The woman should be informed that this discussion will take place.

5.1.2 Clinical Midwifery Manager

Following discussion with the midwife, if appropriate, a meeting should be arranged with the woman. The purpose of this meeting is to ascertain the woman's wishes and have a comprehensive discussion of any alternatives choices that may be acceptable and the risks involved if routine care is declined.

Following this meeting, the woman should be sent a letter documenting the discussion and including an agreed Plan of Support. This should form part of the woman's EPR. A copy should also be shared with: the Named Midwife, Named Consultant Obstetrician, and Maternity Managers.

5.2 Labour in hospital

5.2.1 The Midwife providing care

If a plan of care was made during pregnancy, ensure you are familiar with the woman's choices and discuss with her if she would like to make any changes.

- Inform the Labour Ward Co-Ordinator and Specialist Registrar on duty
- Request that the Labour Ward Co-Ordinator and / or Specialist Registrar review the woman and document all discussions and make a plan of care which reflects the plan made during pregnancy unless the women has changed her mind. This should be clearly documented within the EPR.
- If the woman declines to see the Labour Ward Co-Ordinator / Specialist Registrar, the midwife should seek guidance about information to be given to woman and ensure this is clearly documented within the EPR. It is advisable to have another

member of staff present to witness this discussion and this should be reflected within the documentation.

- If concerns regarding fetal or maternal wellbeing arise during labour, communicate these concerns clearly to the woman including what action you would advise be taken, in order to enable her to continue to make informed choices about her care.
- Ensure Labour Ward Co-Ordinator / Specialist Registrar are kept updated and informed of any event where the woman declines any other aspects of normal care
- Request second midwife to be present in room for support if needed.

5.2.2 **Labour Ward Co-Ordinator**

- If a plan of care was made during pregnancy, ensure you are familiar with this and any changes that have been made.
- Inform the Senior Midwife on duty or on call.
- Ensure that the Midwife is provided with adequate support, and the Labour Ward Co-Ordinator to be present if needed
- Request to see the woman if she declines to see the Specialist Registrar and document all discussions and advice given together with a plan of care within the EPR.

5.2.3 **Specialist Registrar**

- Review the plan made antenatally (if one was made), then request to see the woman. Discuss the plan of care, including any changes the woman wishes to make, then document all discussions and advice given together with a revised plan of care within the EPR.
- Inform the Consultant Obstetrician on duty or on call, if necessary.
- Inform the Neonatal Specialist Registrar, if appropriate.

5.2.4 **Consultant Obstetrician on duty / on call**

- Provide support to the Specialist Registrar and review all discussions and advice to be given and help formulate plans of care
- Attend if appropriate.

5.2.5 **On Call Midwifery Manager**

- Provide midwifery support and advice to the Midwife providing care and the Matron if required.
- Attend if appropriate or requested.

5.3 **Women in labour at home with Plan of Support**

- Discuss plan with woman and check that she consents to care according to the plan that was made.
- Follow plan
- Inform Labour Ward Co-Ordinator and ensure she is aware of the agreed plan of care.
- Inform Midwifery Manager on call only if additional concerns arise or support is required

5.4 Women in labour at home (without Plan of Support)

- Identify risk factors and establish if transfer to obstetric unit is indicated or if labour can safely continue at home.
- If transfer is indicated, discuss rationale with woman and document discussion.
- Call an ambulance
- Inform Labour Ward Co-Ordinator
- If woman agrees to transfer, ensure appropriate handover of care to obstetric team, sharing all relevant information.

5.5 Women in labour at home who decline transfer to hospital

- Continue to provide care in accordance with woman's choice having documented all risks, concerns and advice given.
- Inform Labour Ward Co-Ordinator and request second midwife if appropriate.
- Inform Midwifery Manager on call only if additional concerns or support required

5.6 Parents refusing unwell baby/child admission to hospital

- Health professionals have a duty of care to the baby once born
- Continue to provide care, and outline the risks to the parents
- Document all conversations within the EPR
- If parents continue to refuse admission and you feel the baby requires medical review then see Appendix 1 – Flowchart for Midwife Concerns About a Child.

5.7 Actions to be taken if a woman asks midwife to leave her home

- Legally midwife must leave if asked to do so – (ensure woman is asking you to leave her home and not just a room)
- Take all equipment from the home
- Ensure woman is informed that she can call and ask a midwife to re-attend her home at any time.
- Inform the Midwifery Manager on call and Labour Ward Co-Ordinator.
- Clearly document all discussions held within the EPR.
- Discuss the risks of no professional being present at birth:
  - Risk of injury or death to mother and / or baby
  - Possible child protection issues and legal implications to her and any other attending lay person involved

6. SPECIFIC ADVICE TO BE GIVEN

Women should be given the risks, benefits and recommendations in a non-threatening, non-coercive way, in order for them to make an informed choice. Factual information and, where possible, statistics should be provided to help women understand the level of risk. Absolute numbers should be used rather than relative risk. Both positive and negative framing should be used to help women to understand the risks or benefits of actions, for example, a 3 in 100 chance of a treatment being unsuccessful means a 97 in 100 chance of it being successful.

6.1 Fetal monitoring

6.1.1 Ideally, a discussion around fetal monitoring should take place during pregnancy with a clear plan documented prior to the onset of labour. On admission in labour, this should be discussed with the woman to ensure she is still happy with the plan that has been documented.

Women refusing any form of fetal auscultation should be clearly informed of the following and the discussion and responses clearly documented within the EPR.

'Without monitoring we are unable to identify whether your baby is distressed which could result in your baby being born in poor condition with lifelong health consequences or even in the death of your baby. We would therefore need to recommend that your baby is delivered by Caesarean section as soon as possible'.

6.1.2 A list of criteria where continuous fetal monitoring is recommended can be found in the Guideline for Electronic Fetal Monitoring. Women with identified risk factors who decline continuous electronic fetal monitoring (EFM) should be informed that monitors using telemetry are available to facilitate mobilisation and upright birth positions. If, after discussion, this is still declined then advise and document the following:

'Continuous fetal monitoring is the best available method for ascertaining fetal wellbeing during labour. Without it we may be unable to identify whether your baby is distressed which could result in your baby being born in poor condition with lifelong health consequences or even in the death of your baby. We would therefore need to recommend that your baby is delivered by Caesarean section as soon as possible'.

6.1.3 For women who decline continuous fetal monitoring when undertaking a VBAC, this additional information should be discussed and documented within the EPR:

'Continuous CTG can identify changes in the fetal heart rate more quickly than intermittent auscultation, which may enable prompt identification and management of scar rupture'.

6.1.4 Women declining continuous fetal monitoring and Caesarean section should be offered intermittent fetal monitoring but also informed that:

'Intermittent monitoring for women with risk factors may not identify that your baby is in distress which could result in your baby being born in poor condition with lifelong consequences or even in the death of your baby'.

6.2 Women who decline continuous EFM should be offered regular 20-30 minute EFM every 2 hours with intermittent fetal heart auscultation between. It should be made clear that continuous EFM will be recommended if any abnormalities are identified – See Clinical Guideline for Electronic Fetal Monitoring.

6.3 If women with risk factors choose intermittent fetal heart auscultation they should be made aware that continuous EFM will be recommended if any abnormalities are heard. – See Clinical Guideline for Electronic Fetal Monitoring.

6.4 Routine AN bloods

If woman decline to have their blood taken during pregnancy, a discussion around the reason for this should be had. This would enable the midwife to ascertain if it is due to needle phobia and then to explore if this issue may be overcome during pregnancy. Women should be reassured that they can choose to have their bloods taken at any point in pregnancy if they change their mind.

If, after discussing with the women the rationale for each of the blood tests recommended during pregnancy, she declines then the discussion should be clearly documented within the EPR, including that she is aware of the following issues:

- The risk of miscarriage in the next pregnancy if Rhesus negative status is not detected.

- The potential delay in treatment in an emergency situation if no blood results are available on the hospital system which could lead to maternal compromise or even death.
- The potentially missed opportunity to treat infectious diseases in the mother and take steps to reduce the risk of transmission to the baby.

It is important that a Family Origin Questionnaire is completed even for women who decline to have their blood tested.

6.5 Women who decline IV access in labour

Where risk factors are present and a prophylactic cannula is recommended but declined, the reasons for this should be explored to see if a compromise can be found (eg. Use of local anaesthetic for insertion).
If the woman declines a prophylactic cannula following this discussions, they should be informed of the rationale for this ie. to facilitate rapid fluid replacement or drug administration in an emergency situation which may be delayed if a cannula is not inserted until an emergency occurs – with the possible consequences of maternal compromise or death.

7. CONCLUDING STATEMENT

The role of the midwife is to provide safe, evidence based care to women and their babies. However, this role also includes being an advocate for women and even when women make choices that are not in line with Trust guidance, these must be respected.

Maintaining positive relationships with women who decline routine care can help midwives to reach compromises whereby the risks to mother and baby can be reduced whilst still facilitating informed choice.

Clear and consice documentation of all discussions and referral to senior staff for support and advice are essential in circumstances where compromise cannot be found.

8. REFERENCES

Nursing and Midwifery Council *2018,The code: Professional standards of practice and behaviour for nurses, midwives and nursing associates* London: Nursing and Midwifery Council

Royal College of Obstetricians and Gynaecologists 2015, Birth After Previous Caesarean section: Green top guideline 45 London: RCOG

9. ASSOCIATED TRUST GUIDELINES

Auscultation of the fetal heart guideline

Electronic fetal monitoring guideline

10. PUBLICATION DETAILS

Author of Clinical Guideline	Matron
Division/ Department responsible for Clinical Guideline	Specialist Services/CWH/Maternity
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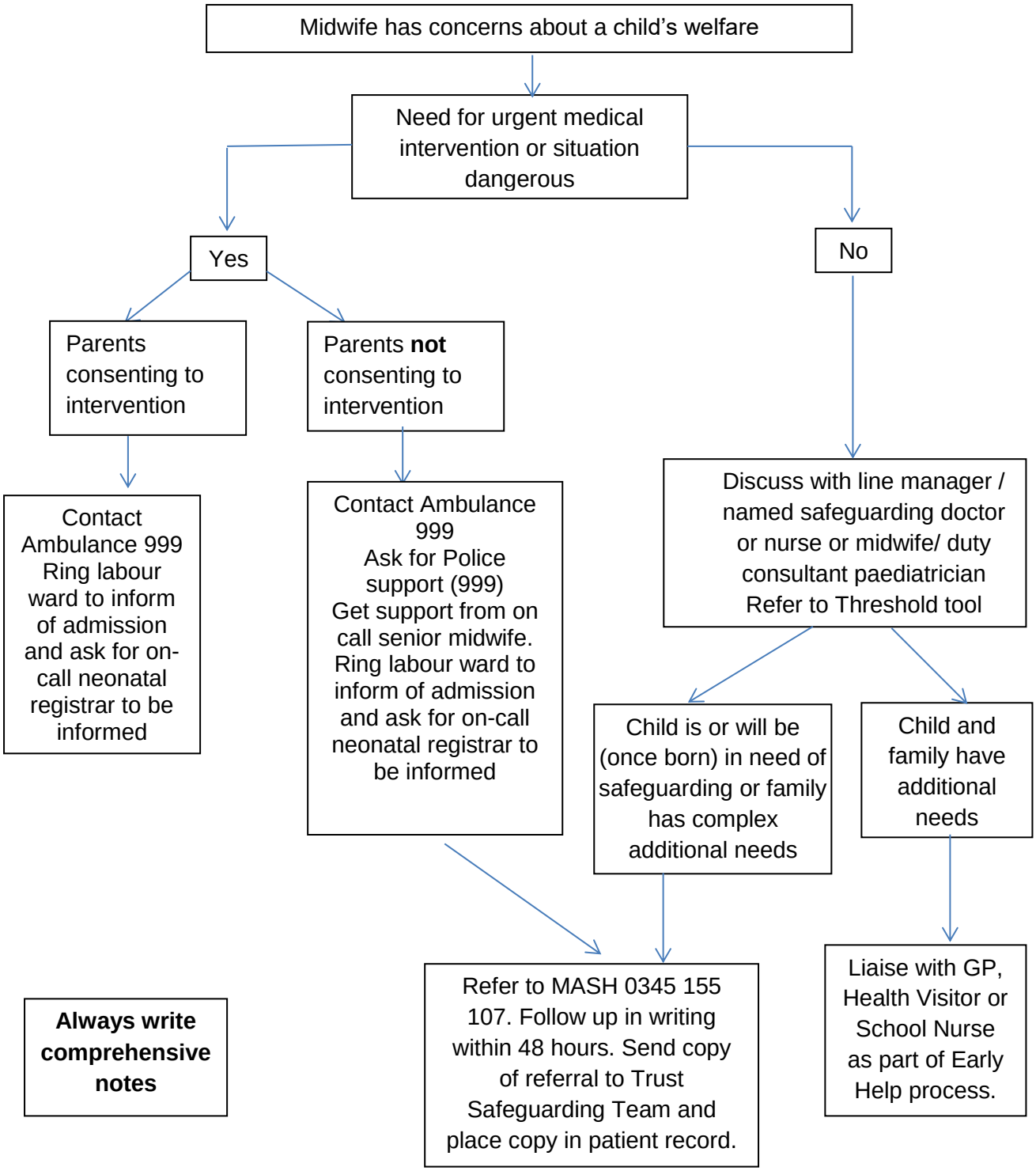
APPENDIX 1 – Flowchart for Midwife Concerns About a Child



If you believe that a baby or child requires medical intervention, discuss your concerns with the parent including the risks and implications for the child. Document these discussions within the EPR. If the parent refuses medical intervention you will need to consider the rights of the child. If a child is likely to suffer significant harm if it does not receive medical intervention or if it were to remain in a dangerous situation, this would override a parent's wishes and would justify you taking action without their consent (Children's Act 2004).